

Alcoholism

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ALCOHOLISM

Remember that we deal with alcohol—cunning, baffling, powerful!

—ALCOHOLICS ANONYMOUS

ALCOHOLISM IS A DISORDER of great destructive power. Depending on how we define it, it afflicts between 6 and 20 percent of all Americans at some time in their lives. In the United States, alcoholism is involved in a quarter of all admissions to general hospitals, and it plays a major role in the four most common causes of death in twenty-to forty-year-old men: suicide, accidents, homicide, and cirrhosis of the liver. 1 The damage it causes falls not only on alcoholics themselves but on their families and friends as well—and this damage touches one American family in three. Life is not a cog railway that we step onto at birth and off at death, secure in the knowledge that we are safe from accidental derailments and the tug of gravity. No matter how blessed by good fortune we start out or how blighted by its lack, our circumstances can always change, and so can the conditions under which we meet them. This is the Study's sixth lesson, and much of what we learned about it came from our prospective longitudinal investigation of alcohol use and abuse.

The Grant Study's involvement in alcohol research was one of the silver linings of our perpetual anxiety about funding, and true sterling. Without it, I would not have been forced into this last ten years' reexamination of marriage, divorce, and the development of intimacy. Yet that reexamination called into question not only some cherished assumptions of my own, but also assumptions that predated my tenure at the Study, and the received wisdom of several generations. Lifetime studies are bread cast upon the waters. You can't know in advance everything you should be finding out. But on the other hand, some of what you find out and have no idea what to do with may turn out to be invaluable unforeseen years later. The work with alcohol was like that.

It isn't easy to identify who is and is not an alcoholic. Until now, most major longitudinal studies of health (for example, the Framingham Study in Massachusetts and the Alameda County Study in California) have taken into account only alcohol consumption, not alcohol abuse. 2 Unfortunately, as I've said before, reported alcohol consumption identifies alcohol abuse almost as poorly as reported food consumption reflects obesity. In contrast, the Grant Study has always focused on alcohol-related problems. Where alcohol is concerned, it is what people do, not what they say, that is important.

Our study of the College and Inner City cohorts is the longest and most thorough study of alcohol abuse in the world. It has established answers to seven major questions:

1. Is alcoholism a symptom or a disease?
2. Is alcoholism environmental or genetic?
3. Are alcoholics premorbidly different from nonalcoholics?
4. Should the goal of alcohol treatment be abstinence?
5. Can "real" alcoholics ever drink safely again?
6. How can relapse be prevented?
7. Is recovery through AA the exception or the rule?

In several instances, the Study's longitudinal findings differ from those of well-respected cross-sectional studies.

METHODS

The Study's unique structure gave it three advantages as we made our usual effort to replace opinion with science. First, the men were followed for their entire lifetimes—a rarity but a necessity, because alcoholism is a relapsing and evolving disease. Second, the Study quantified alcoholism not by reports of quantity or frequency of drinking, but by objective numbers of alcohol-related problems. Third, over its own lifespan the Study has had between thirty and fifty contacts with each of its members, which greatly facilitated the collection of this data.

A few notes on how the men were studied. The questionnaires they received every two years asked if they, their friends, their families, or their physicians had expressed concern about their drinking, and whether and for how long they had ever stopped drinking (evidence not of control, but of loss of control). At interview, alcohol abuse or its absence was always specifically recorded. When the men reached forty-seven, 87 percent of them participated in a two-hour semi-structured interview with a detailed twenty-three-item section on lifetime problem drinking.³ Since forty-seven, they have undergone physical examinations every five years. Any man not previously classified as an alcohol abuser who answered yes twice in a row to two or more of the four concern questions, or who through interview or telephone contact acknowledged alcohol abuse, or whose physical exam revealed evidence of alcohol abuse, was classified an alcohol abuser. The age that each participant first met DSM-III criteria for alcohol abuse was estimated from all available data: questionnaires, any relevant court records, social service data, family interviews, etc.

(In 1962, before the Inner City cohort joined the Harvard Study of Adult Development, arrest records and records of psychiatric hospitalization, if any, had been searched for more than 95 percent of the men, who were at high risk for alcoholism, and for the preceding two generations of their families.⁴ These are data that are well-nigh irreplaceable, as recent privacy legislation now precludes such searches.)

By mining interviews, clinical data, objective documents, and self-reports obtained from the participants by clinicians experienced in treating alcoholism, we were able to establish both a categorical and a dimensional scale of alcohol use for the men of both cohorts. The categorical scale was derived from DSM-III, the third edition of the Diagnostic and Statistical Manual of the American Psychiatric Association; that was the version current in 1977–1980, when this analysis was taking place.⁵ It distinguished three types of alcohol use: social drinking (that is, no chronic problems related to alcohol use), alcohol abuse (chronic problems but no physiological dependence), and alcohol dependence (presence of withdrawal symptoms or hospitalization for detoxification). In this chapter, I will use the term alcoholism to refer to both of the latter two categories.

The dimensional scale, the Problem Drinking Scale, or PDS, assessed problem drinking on a continuum of severity by means of sixteen equally weighted questions (similar to those of the Michigan Alcoholism Screening Test).⁶ The PDS inquired about social, legal, medical, and job problems caused by alcohol abuse. It also asked about blackouts, going on the wagon, seeking treatment, withdrawal symptoms, and problems with control. Scores of 4–7 on the PDS usually met the DSM-III criteria for “alcohol abuse,” and scores of 8–12 usually met the criteria for “alcohol dependence.” Men with fewer than 4 lifetime problem points on the dimensional scale were usually classified as social drinkers.

Observant readers will note that the numbers in this chapter vary sometimes from those in older reports. This is because in those earlier analyses we included all the Study men who had ever met DSM-III criteria for alcohol abuse (153 out of the 456 original Inner City men and 56 of the original 268 College men). In refining the original analysis for this book, however, men whose problem drinking scores were borderline (3 or 4) and who abused alcohol for less than five years and who returned to social drinking for the rest of their lives (13 Inner City and 2 College men) were reclassified as social drinkers.

We have excellent and inclusive death data, including death certificates for all Study members, including those who withdrew, except for two who died abroad. Survival or mortality has been ascertained and documented through the National Death Index or credit agencies, whichever was applicable, and death certificates obtained. Data from death certificates, and recent physical examinations from participating Study members, were used to infer major causes of death.

We assessed the alcohol status of all cooperating Study members, whether or not they had been alcoholic in the past, every year between the ages of twenty and seventy, using the biennial questionnaires and triangulating them with other material. (Since the Glueck men had not been personally followed between thirty-two and forty-five, for those years we had to depend on history and public records, including records of arrests.) We categorized the alcoholics as follows:

Abstinent: less than one drink (0.5 ounces of ethanol) a month for a year. Return-to-controlled-drinking: a former alcohol abuser consuming more than one drink a month for at least three years with no reported problems. Continued alcohol abuse: Clear past history of sustained alcohol abuse and one or more acknowledged problems caused by drinking in the past three years. When data was missing for three years, the yearly status was rated as unknown. Data on alcohol abuse for every man was obtained between twenty and forty times over sixty (on average) years of observation. Nonresponders for two consecutive questionnaires were interviewed in person or by telephone. Men were classified as Dropped if they asked to withdraw or ignored questionnaires and follow-up telephone calls for ten years.

Figure 9.1 Final alcohol outcome status.

Figure 9.1 reflects the men's alcohol status at age sixty. ⁷ In the case of death or institutionalization, outcome status was based on the last three years of community residence prior to death or chronic institutional care.

By age seventy (approximately 1990 for the College men and 2000 for the Inner City men), 65 (46 percent) of the 140 Inner City alcoholics and 19 (35 percent) of the 54 College alcoholics were dead. (Recall that under "alcoholics" I am subsuming both alcohol-abusing and alcohol-dependent men.) In 2003, we found that all but 3 of the 18 alcohol-dependent College alcoholics had died by age eighty, and that their lifespans on average had been seventeen years shorter than those of their social-drinking Study peers.

Figure 9.1 illustrates vividly why the prevalence of alcoholics goes down with time. The problem isn't "burnout" (as AA members put it, getting "sick and tired of being sick and tired"); this is rare among alcoholics. Nor is it poor case-finding among the elderly. It's that over time, alcoholics become abstinent or die.

Active alcoholics die twice as fast as abstinent alcoholics, but even the latter die significantly sooner than social drinkers—often because cigarette abuse continues after alcohol consumption stops. As we will see, it is only for the first ten years of abusive drinking that the diagnosis of alcoholism is ambiguous. Over decades, alcoholism becomes a black-or-white disease.

THE SEVEN QUESTIONS

1. Is alcoholism an evanescent symptom or a chronic disease? Social scientists generally consider matters such as intelligence, drinking behavior, and even eyesight as continua; physicians in the trenches have little patience for such nuances. Who is right? Both. The definition of retardation, alcoholism, or blindness may depend upon a host of independent contextual, interpersonal, and motivational factors. But when an alcoholic actually goes to the expense and inconvenience of seeking help for a self-acknowledged disease, there tends to be a certain crispness of definition as to what the problem is.

The medical model of alcoholism is one of inexorable progression. It was made famous by William Hogarth in the series of paintings known as *The Rake's Progress*. ⁸ It has been retrospectively documented by E. M. Jellinek. ⁹ It is taken as an article of faith by Alcoholics Anonymous. ¹⁰ Yet how can this model be reconciled with the unpredictable oscillations observed in fine-grained prospective studies of alcoholics? ¹¹ Short-term prospective investigations indicate that during any given month a majority of alcoholics are either abstinent or drinking asymptotically. This cannot be said of either cigarette or heroin addicts. Is alcoholic progression therefore a myth? When does the state of drunkenness (which is often voluntary) become the trait of chronic problem drinking (which is largely involuntary)? The Study helped to clarify.

Virtually every alcohol abuser in the Study, no matter how chronic, had periods of abstinence lasting a month or longer. In fact, a history of going "on the wagon" is a commonly accepted criterion for the diagnosis of alcoholism. The more physiologically dependent and symptomatic the alcoholic, the more likely he or she is to have attempted abstinence, usually more than once. As I've said elsewhere, Mark Twain found it so easy to stop smoking that he did it twenty times.

That is why it is only the number and frequency of alcohol-related problems that can truly define the clinical phenomenon known as alcoholism. Alcoholism is a construct of a higher order of complexity than mumps or retardation. It does not reflect a specific pathogen, like mumps. It is like retardation in being another diagnosis that depends on where a clinician draws the line. But more than anything else, like Type II diabetes, hypertension, and coronary artery disease, the disease of alcoholism is the endstage effect of bad habits on facilitating genes.

Although its symptoms may come and go, alcoholism acts like a chronic disease, and chronic diseases are forever. Without specific treatment, diabetes will plague you until you die—young. The Harvard Study of Adult Development's seventy-five years have enabled us to document that without sustained abstinence, the vast majority of problem drinkers continue to have alcohol-related problems until they, too, die young. Seventy-two percent of the College social drinkers lived to be eighty, but only 47 percent of the College alcohol abusers and a minuscule 14 percent of the alcohol-dependent College men—a very significant decline.

On the one hand, the Study demonstrated that alcohol abuse is inexorably progressive only in its initial stages. Once a drinker's alcohol consumption has gotten “out of control” and become a source of problems, it can remain so for a lifetime without necessarily progressing to morning drinking, job loss, or even severe withdrawal symptoms. Seven of the eighty-year-old College alcohol abusers had misused alcohol for decades (about thirty years, on average), but without evidence of worsening symptomatology. Similar courses can be seen in cigarette dependency.

On the other hand, alcoholism does not get better. Those same seven men over those same thirty years continued to report alcohol-related problems affecting their self-esteem, their health, and their families. In this, alcoholism does conform to the conventional disease model, and here too there is a resemblance to cigarette dependency. Two-pack-a-day smokers rarely revert to half-a-pack-a-day social smoking, and once an alcoholic has developed the problems typical of dependence, he or she is unlikely to revert to social drinking or even abuse. More on this in a moment.

Thus I think it is both appropriate and useful to consider alcoholism a disease. The diagnosis is not made so lightly that social drinkers are likely to suffer from incorrect labeling. Nor are the manifestations of alcohol abuse so varied as to render a unifying diagnosis meaningless. There is a benefit in calling severe problem drinking a disease; alcoholics who label themselves “ill”—as opposed to “bad”—will feel less helpless; they will have higher self-esteem; and they will likely try harder to change and to let others help change them.

A final reason to consider alcoholism a disease is that it kills people—tens of thousands of people a year. By age eighty, a College alcoholic was twice as likely to be dead as a nonalcoholic. By age seventy, almost half (46 percent) of the alcohol-abusing Inner City men were dead, as opposed to 29 percent of the nonabusers. Admittedly, much of this increased mortality does not reflect direct physical effects of alcohol itself, but it does point up graphically that cigarette consumption among problem drinkers was vastly greater than that of social drinkers. The average nonalcoholic smoked for fourteen-pack years (half a pack a day for twenty-eight years or two packs a day for seven years would both equal fourteen-pack years). But the average alcohol abuser smoked for twenty-seven-pack years, and the average alcohol-dependent individual for fifty—more than three times the cigarette consumption of social smokers. There is no evidence that heavy smoking increases drinking. However, as of 2010, one College alcoholic in four had died of heart disease. Of the social smokers, only one in eight had. Three percent of the College social drinkers died of lung cancer; 15 percent, five times as many, of the alcoholics did. Similarly, Inner City alcoholics were twice as likely to die from lung cancer as social drinkers.

The overlap between drinking and smoking does not indicate the presence of an “addictive” or “oral” personality or any other such abstraction, only the concrete reality that conscience and judgment are soluble in ethanol. Alcoholics are fearless in barrooms and automobiles. They're not careful about safe sex, and they don't worry about cigarettes, either. Deaths from cirrhosis, accident, suicide, and pharyngeal cancer were also far more common among alcohol abusers. Study findings in this regard are very similar to those from eight other longitudinal studies of premature mortality in alcoholics reviewed elsewhere. 12

2. Is alcoholism environmental or genetic? In 1938, the year the Grant Study began, Karl Menninger, America's most famous psychiatrist, made a bold statement: “The older psychiatrists . . . considered alcoholism to be a hereditary trait. Of course, scarcely any scientist believes so today. Although it's still a popular theory, alcoholism cannot possibly be a hereditary trait, but for a father to be an alcoholic is an easy way for a son to learn how to effect the retaliation he later feels compelled to inflict.” 13

Menninger was wrong. When it comes to alcoholism, genes trump environment. Our data revealed that a family history of alcoholism more than doubled the chance that a Study member would become alcoholic, even when all other suspected etiologies (such as ethnicity, social class, and multiple family problems) were statistically controlled. But children with alcoholic stepparents were not more likely to become alcoholic. Here the data from the Inner City men was

even more convincing than that from the Grant men.

Until very recently, most social scientists believed unhappy childhoods to be a cause of alcoholism. Our data showed that the association of childhood environmental weaknesses with future risk of alcohol abuse exactly paralleled the extent of parental alcohol abuse: that is, the more severe a parent's alcoholism, the more it will be reflected in his child's environment, and in the severity of alcoholism in the child. Contrary to Menninger's belief, however, alcoholism does not arise in children in response to an unhappy childhood or even to life with an alcoholic stepparent. It is the fact of an alcoholic biological parent, whether or not the child lives under the same roof, that increases the child's risk. Of the fifty-one men who had few childhood environmental weaknesses and an alcoholic parent, 27 percent became alcohol-dependent. Of the fifty-six men with many environmental weaknesses and no alcoholic parent, only 5 percent became alcohol-dependent. Alcoholic parents do not have to live with their children to pass on the disease.

The presence of a genetic component does not free us from issues of nature and nurture. While the number of alcoholics in one's ancestry increases the likelihood of alcohol abuse for genetic reasons, it also increases the likelihood of lifelong teetotaling, presumably for environmental reasons. Almost half of the forty-eight teetotalers of Anglo-Irish-American descent in the Glueck Study had an alcoholic parent.

3. Are alcoholics premorbidly different? This question essentially explores whether alcoholism is a symptom of mental illness or a cause. It has long and widely been considered the former. In the year the Grant Study was conceived, Robert Knight, a prominent psychoanalyst at the Austen Riggs Center, said it directly: "Alcohol addiction is a symptom rather than a disease. . . . There is always an underlying personality disorder evidenced by obvious maladjustment, neurotic character traits, emotional immaturity or infantilism." 14 In 1940 Paul Schilder, an Austrian psychiatric researcher who has given his name to four diseases, concurred. "The chronic alcoholic person is one who from his earliest childhood on has lived in a state of insecurity." 15 Two decades later, E. M. Jellinek, Yale's great alcoholism scholar, wrote: "In spite of a great diversity in personality structure among alcoholics, there appears in a large proportion of them a low tolerance for tension coupled with an inability to cope with psychological stresses." 16 And in 1980, psychiatrist Michael Selzer wrote more generally in the leading textbook of psychiatry: "Despite occasional disclaimers, alcoholics do not resemble a randomly chosen population." 17 None of these world-renowned experts, however, possessed any prospectively gathered data as to what alcoholics were like before they became alcoholic.

Three premorbid personality types have repeatedly been proposed as causal contributors to alcoholism: the dependent, the depressed, and the sociopathic. 18 The Grant Study confirmed none of these hypotheses.

The College alcohol abusers did not exhibit more premorbid evidence of dependent personality disorder than men who remained social drinkers all their lives, and 58 percent of the College men who became alcohol abusers did not lose control of their alcohol use until after age forty-five.

There were men who displayed many dependent traits as young adults and who showed lifelong difficulties with loving, with perseverance, and with postponement of gratification. Such so-called oral-dependent men were also more anxious and more inhibited about expressing aggression. Yet these traits in young adulthood were not significantly more common in future alcohol abusers than in everyone else. Oral-dependent traits did become very common in the College men, however, once they began to abuse alcohol.

It is also true that alcohol-abusing College men were five times more likely to report severe depression than men who did not abuse alcohol. Furthermore, of the thirty-one men who ever appeared to manifest major depressive disorder, fourteen (44 percent) also manifested alcoholism. Following these men for twenty-five years, I received the impression that many of the fourteen had turned to alcohol to relieve their depression. In 1990, however, the longitudinal data were subjected to blind analysis, and that impression proved to be illusion. 19 One psychiatrist, blind to age of onset of depression, reviewed each man's entire record and estimated the year that he first manifested evidence of alcohol abuse. A second psychiatrist, blind to age of onset of alcoholism, reviewed each man's record and determined the age of onset of major depressive disorder (or probable major depressive disorder). In four of the fourteen cases, the psychiatrist looking for evidence of primary depression believed that the depressive symptoms could be explained entirely by alcohol abuse. In six more cases, the rater noted that the first episode of major depressive disorder had occurred (on average twelve) years after the patient met the criteria of alcohol abuse. In only four cases had a man's depression actually preceded his alcoholism. Given the prevalence of alcoholism and affective disorder among the 268 men in the College sample, chance

alone could account for primary alcoholism and primary depression occurring independently in four cases or even more.

As for the sociopathic personality, alcoholics are somewhat more likely to be premorbidly antisocial than asymptomatic drinkers. 20 And some antisocial adolescents initiate alcohol abuse as their antisocial behavior develops. But most alcoholics are not premorbidly antisocial; they become antisocial only after developing alcoholism. The role of sociopathy in alcoholism remains murky and I've discussed it extensively elsewhere. 21

Two of the three supposed personality antecedents of alcoholism having been dispatched, and the third on hold for the time being, we looked for more general premorbid factors that might predict later alcohol abuse. However, data from the College sample revealed that neither bleak childhoods, childhood psychological problems, nor (more positively) psychological stability in college differentiated future social drinkers from future alcohol abusers.

Far more surprising, most future alcoholics do not appear to differ from future asymptomatic drinkers even in terms of premorbid psychological stability. A hypothesis like that could not even be seriously entertained until prospective studies were available, so compelling was the illusion that unhappy, nervous people turn to alcohol for self-medication, and so unlikely did it seem that depression and the so-called alcoholic personality might be secondary to the disorder of alcoholism. But alcohol in high doses is neither a stimulant nor a tranquilizer; it is the very opposite of a successful drug for these conditions. And alcohol ingestion makes both insomnia and depression worse. 22

Table 9.1 Correlation of Alcoholism and Dependent Traits with Premorbid Variables

Very significant = $p < .001$; Significant = $p < .01$; NS = Not Significant.

In the Inner City sample, the three childhood variables that most powerfully predicted positive adult mental health—warmth of childhood, freedom from childhood emotional problems, and boyhood competence—did not predict freedom from alcoholism; similarly, the three variables that most powerfully did predict alcoholism—family history of alcoholism, ethnicity, and adolescent behavior problems—did not predict poor future mental health. Alcoholism and poor mental health cannot be yoked invariably together.

In dismissing these factors (unhappy childhoods, multiproblem families, depression, and anxiety) as major causes of alcoholism, I do not mean to say that they are not important. They are always important, and they will make any chronic disease worse. I simply wish to underscore that in a prospective design, when more salient variables like culture and familial alcoholism can be controlled by the choice of sample, premorbid personal and family instability do not make a statistical contribution to the risk of alcoholism. Let me also reiterate that the Inner City men with an alcoholic biological parent but an otherwise stable family were five times as likely to develop alcoholism as men from clearly multiproblem families but without an alcoholic biological parent.

Our data suggested only two areas in which alcoholics appeared to be premorbidly different from asymptomatic drinkers. Before they become abusers, future alcoholics can tolerate relatively larger amounts of alcohol than controls can without hangovers, vomiting, or staggering. 23 This difference appears at least in part to be another reflection of the hereditary component of alcoholism.

The second difference is that future alcoholics are more likely to come from environments that tolerate adult drunkenness and discourage children and adolescents from learning safe drinking practices—Ireland and the United States are prime examples. Alcoholics were less likely to come from cultures that permit adolescents to consume low-proof alcoholic beverages at ceremonies and with meals, but condemn drunkenness—Italy, for example. Thus, the parents and grandparents of the alcoholics in our samples were several times more likely to have been born in English-speaking countries than in Mediterranean ones. Controlling for genetic risk, the Irish Study members had very significantly higher rates of alcohol dependence than the Italians. One highly controversial conclusion of the Study is that alcoholism is more affected by parental modeling of how children should drink than by legislation of when children should drink.

4. Should the goal of treatment be abstinence? When the Grant Study began, we knew far more about the natural history of most cancers than we did of alcoholism. Even the clinical course of alcoholism was poorly understood. What actually happens to alcoholics over time—not just those who attend clinics, but the whole constellation of treated and untreated alcoholics? Why does the prevalence of alcohol abuse decline sharply with age? 24 Is the explanation for this

decline “burnout” (no), or return to asymptomatic drinking (no), or stable abstinence (yes), or high mortality (yes)? Another question is how long abstinence or return-to-controlled-drinking must persist before an individual’s recovery from alcoholism can be considered secure. In smoking and even in cancer, remission must usually endure for five years before relapse is considered unlikely. In alcohol treatment studies, however, investigators often speak of “recovery” after the drinker has been symptom-free for six months or a year. The Harvard Study of Adult Development demonstrates that such a short time frame is not realistic. Only after five years of abstinence can remission from alcoholism be regarded as secure. 25 And as with smoking, the remitted alcoholic can almost never return to social drinking.

Table 9.2 summarizes the outcome status for the 148 alcoholics from the College and Inner City cohorts successfully followed until death or age seventy. It clarifies why the numbers of alcoholics seem to decline with age, and why at age seventy only a quarter of the men were still abusing alcohol. Some had been stably abstinent (for a mean of nineteen years), and a few had returned to social drinking, less stably. But almost half of them were dead.

An interesting finding: the alcoholic Inner City men were more likely to become abstinent than the alcoholic College men. Only nine of the College men meeting criteria for alcohol abuse achieved three or more years of abstinence, while 51 alcoholic Inner City men achieved this status.

5. Can “real” alcoholics ever drink safely again? The answer to this question is “Yes, but . . .” My hesitation is based on four caveats: one from the general literature, and three based on Study findings. In fifty years of alcoholism literature, every study that reported the successful return of its clients to social drinking (often making the evening news in the process) has been found to have been in error after ten years’ follow-up. 26 One highly publicized cautionary tale was the case of Audrey Kishline. Kishline founded Moderation Management in 1994, but in March of 2000, she swerved out of her lane and killed two occupants of an oncoming car in a drunken driving accident. 27

Table 9.2 Outcome Status for Study Alcoholics at Age 70 or at Time of Death

Second, our data revealed that of the men who returned successfully to social drinking, most had only barely met the criteria for a diagnosis of alcoholism. This was true in both the Inner City and College samples. Third, of those who had maintained social drinking for three years or more, half relapsed or ended up seeking abstinence. Fourth, even the successes (the men who at last follow-up have had no further problems due to alcohol) often found it impossible to return to the carefree use of alcohol manifested by most social drinkers.

The year 1977 was the end of the age twenty-to-forty-seven follow-up period for the Inner City men. Twenty-one of them had achieved stable abstinence of three years or more, and twenty-two had returned to controlled drinking for three years or more. 28 Continued follow-up until 1992 revealed that eighteen (86 percent) of the twenty-one abstinent men maintained their abstinence until age sixty or until death. 29 The range of known abstinence for these eighteen men was three to thirty-seven years; the mean length of abstinence was twenty years. In contrast, over the same fifteen-year follow-up period, seven of the twenty-two men with three or more years of controlled drinking—almost a third—relapsed to sustained alcohol abuse, with a mean duration of alleged controlled drinking before relapse of twelve years. Three of the twenty-two sought sustained abstinence, four withdrew from the Study, and three, because of the brevity of their abuse, were reclassified as non-alcoholic. Thus, in 1992 or at death, only five of the twenty-two men were still believed to be true alcoholics drinking in a controlled fashion. Figure 9.2 illustrates the relation of the number of alcohol-related problems to the likelihood of a return to social drinking.

Figure 9.2 Relation between number of alcohol-related problems and likelihood of a return to social drinking.

6. How do we prevent relapse? Observers of human nature have long puzzled over the phenomenon of individuals who become “converted” to a new religion or otherwise abruptly alter their life course. The Study sought to discover whether in the case of alcoholism this usually occurred through clinical treatment (no), through willpower (no), through becoming sick and tired of being sick and tired (no), or through nonclinical factors known to affect relapse prevention in other addictions (yes). Alas, going on the wagon is more often evidence of alcoholism than of recovery, just as dieting is more often evidence of obesity than of slimness. Nevertheless, prolonged relapse prevention is key, both in alcoholism recovery and in obesity. Thus, our continued study of two cohorts of alcoholics over fifty years provided a naturalistic way of uncovering who did manage to prevent relapse. 30

In the vast majority of alcoholics, we found that while counseling, detoxification, and even hospitalization could be temporary lifesavers, they did not change the natural history of the disease. As with diabetes and obesity, permanent changes in self-care are the only way to extend life over the long run. Table 9.3 covers four well-established means of relapse prevention in any addiction. Stably abstinent Study members on average employed two out of four of them during their first year of abstinence. About half of the ever-abstinent men found an alternative for alcohol, and some found more than one. Substitute dependencies varied from candy binges (five men) to benzodiazepines like Valium or Librium (five men); from compulsively helping others (two men) to returning to dependence upon parents (two men); from marijuana (two men) to mystical belief, prayer, and meditation (five men); from compulsive work or hobbies (nine men) to compulsive gambling (two men); from compulsive eating (three men) to chain smoking (seven men).

Table 9.3 Factors Associated with Abstinence for a Year or More for 49 Untreated and 29 Clinic-Treated Inner City Men

Another half of the men achieved relapse prevention through compulsory supervision or behavior modification. By that I mean the presence of factors independent of willpower that systematically altered the consequences of alcohol abuse. This was often probation, or a painful medical consequence that, as AA members say, “keeps the memory green.”

Two other means of relapse prevention were employed by almost half the men. One was to get involved in an inspirational group (for our sample, usually AA). The second was to find new relationships; sometimes these were love relationships and sometimes opportunities to help needy others. But they were always relationships with people toward whom the alcoholic did not feel guilty for past misdeeds. In their review of the literature on remission from abuse of tobacco, food, opiates, and alcohol, Stall and Biernacki identified these same four factors.³¹ The four factors in Table 9.3 appeared to be the most important keys to relapse prevention; only 30 percent of the men resorted to clinic attendance or hospitalization during their first year of abstinence.

Surprisingly, absence of risk factors for alcohol abuse did not predict successful remission. The skills that get you out of a hole are likely independent of the forces that got you in. The remitted alcoholics abused alcohol for an average of two decades at least, and the severity of their alcoholism and their genetic vulnerability had been if anything greater than that of their nonremitting counterparts. Limited education, which is also a risk factor for alcohol abuse, did not inhibit stable remission. Indeed, the less-educated Inner City men were significantly more likely than their Grant Study counterparts to become abstinent. Although the per capita cigarette consumption of the alcoholics was almost twice that of the nonalcoholics, the severity of cigarette abuse among alcoholics was not statistically associated with eventual abstinence.

7. Is recovery through AA the exception or the rule? For both cohorts, regular AA attendance was strongly associated with sustained abstinence. All four of the factors in Table 9.3 are embodied in the AA program and in many other incorrectly named “self-help” recovery programs organized along similar lines. I put self-help in quotes because AA is as much about self-help as a barn-raising. In both of these community activities, success is at least as much about helping other people as about helping yourself. Four variables were associated with Study men joining AA: severity of alcoholism, Irish ethnicity, absence of maternal neglect, and a warm childhood environment.

Of the nine alcohol-dependent College men who achieved stable abstinence, five (56 percent) attended AA for between 30 and 2,000 meetings. Two other alcohol-dependent College men attended about 50 meetings but relapsed. Of the thirty-nine alcohol-dependent Inner City men with stable abstinence, at least fourteen (36 percent) attended AA for 50 to 2,000 meetings. The Problem Drinking Score assessment of the alcohol-dependent Inner City men with fewer than 30 AA visits was about 9. Men who attended more than 29 AA visits (the mean was 400) had a mean problem drinking score of 12—a very significant difference. One doesn’t usually seek painful hip replacement before one’s arthritis has become quite severe, and “hitting bottom” increases the alcoholic’s willingness to sit on hard church chairs, drink bad coffee, and take “the cotton out of one’s ears and put it in one’s mouth” several times a week. In both cohorts, the men who were stably abstinent attended about twenty times as many AA meetings as the chronically alcoholic (Table 9.4).

Table 9.4 Question: How Does AA Work? Answer: AA Works Fine!

Very significant = $p < .001$; Significant = $p < .01$; NS = Not Significant.

THE STORY OF JAMES O’NEILL

The story of James O'Neill illustrates how alcoholism reverses what are commonly thought to be cause and effect; it demonstrates that alcoholism is the horse in life's troubles, not the cart. O'Neill behaved very badly while drinking, but—however difficult this may be to believe—in 1950, before his alcohol abuse began, he had been assessed by the Study staff as a man of “unqualified” ethical character, and the rather prim director of the Health Services had described him as a “straightforward, decent, honest fellow, should be a good bet in any community.”

James O'Neill did not come to psychiatric attention until 1957, thirteen years after his Harvard graduation, when he was first admitted to a psychiatric ward at a VA hospital. A thirty-six-year-old father of four and former assistant professor of economics, O'Neill described himself as a “failure at his marital and professional responsibilities because of drinking and missing teaching appointments.” His admission note stated, “Present symptoms include excessive drinking, insomnia, guilt and anxiety feeling.” The diagnosis was “behavior disorder, inadequate personality.”

O'Neill provided the following history, paraphrased from his hospital record. He began drinking and gambling in the summer of 1948, while depressed over a poor performance on his Ph.D. generals. He was drinking during the daytime, and missing teaching appointments. However, he did continue to teach and to keep his family together. He finished his Ph.D. without difficulty, and in 1955 he left his large West Coast school for a research university in the South.

At the time of his admission, O'Neill expressed suspicion and anger toward the important people in his life, all of whom, he alleged, had treated him badly. Otherwise he showed little emotion, and the interviewer commented: “His pattern of drinking, sexual infidelity, gambling and irresponsible borrowing led him to recognize from his reading that it adds up to diagnosis of psychopathic personality, especially since he has experienced no real remorse about it.” It was known that he had given his son some books to sell, and that four books from the university library were among them; he was accused of stealing university property, and fired for moral turpitude. He assured the hospital staff that he did not sell university books knowingly.

The psychiatric record continues:

During all of the time that O'Neill was frequenting bars, contacting bookies and registering in hotels to philander, he always used his own name. It's interesting that when he was carrying on his nefarious pursuits, he got considerable satisfaction out of it being known that he was a professor. . . . When his mother died in 1949, he felt no remorse [sic] at her death. He did not remember the year of his mother's death. In view of the fact that he dates his extracurricular activities as beginning in 1948, this confusion is probably significant.

During his eight-month hospital stay, the patient . . . was able to work out a great deal of feelings toward his family, in particular toward his mother and also toward his wife. The patient felt quite hostile and anxious about the fact . . . that his parents were always very cold. . . . He harbored many feelings of hostility toward his wife whom he feels does not appreciate the fact that she's married to such an intelligent college professor. All she wants is to have money and bigger homes.

The discharge diagnosis was “anxiety reaction manifested by feelings of ambivalence about his family, his parents and his work.” The precipitating stress was considered to be “death of the patient's mother and a long history of drinking and gambling and going into debt.” His predisposition was considered to be “an emotionally unstable personality for the past 20 years.” At one point the VA even called him “schizophrenic.” A diagnosis of alcoholism was never even considered.

But the Grant Study record told a completely different story. In college, James O'Neill had been the embodiment of the Grant Study's ideals of optimal health and achievement. He was one of the brightest men in the Study, and after three years of observation he received an A in psychological soundness. A child psychiatrist blind to his life after age eighteen was asked to compare his childhood environment with those of his Grant Study peers. She placed it in the top third, and summarized the raw data on his childhood as follows:

O'Neill was born in a difficult delivery. The mother was told not to have more children. His parents were reliable, consistent, obsessive, devoted parents. They were relatively understanding, and their expectations appear to have been more non-verbal than explicit. The father was characterized as easy to meet, the mother was seen as more quiet; no alcoholism was reported. Warmth, thoughtfulness and devotion to the home were some of the comments. The subject spoke of going to his father first with any problems, but of being closer to his mother than to his father. His peer relations

were reported to have been good, and little or no conflict with his parents was reported.

She went on to predict that O'Neill would develop into "an obsessional, hard-working, non-alcoholic citizen, whose work would be related to law, diplomacy and possibly teaching. He would rely on his intellect and verbal abilities to help in his work. He would probably marry and be relatively straight with his children. He would probably expect high standards from them."

Other observers summed O'Neill up equally favorably in the years before he turned thirty. The dean's office ranked his stability as "A" while he was in college; the Study internist described him as "enthusiastic, whimsical, direct, confidant, no grudges or chips, impressed me as an outstanding fellow." The staff psychiatrist was impressed by his "combination of warmth, vitality and personality," and also put him in the "A" group. When he was twenty-one, he married his childhood sweetheart, with whom he had been in love since he was sixteen; in 1950, six years after they married, the marriage still seemed solid. When O'Neill was twenty-three, his commanding officer wrote that he gave "superior" attention to duty and was a particularly desirable officer.

From the prospective record it was also possible to record a more accurate picture of O'Neill's feelings about his mother's death. The child psychiatrist who assessed the prospective record saw him as among the best mother-child relationships in the Study. His mother's physician commented that O'Neill had been "devoted and helpful during the illness," and in 1950, six months after her death, a Study observer noted that O'Neill felt her loss deeply. It was only seven years later, on his admission to the VA, that O'Neill reported having no feelings toward his mother and blamed her alleged coldness for his current unhappiness. Over time, alcoholics develop excellent collections of "resentments."

O'Neill was one of the lostest of the Study's lost sheep. He had stopped returning questionnaires long before his hospitalization. It was not until 1972 that he finally brought the Study up to date on the progression of his life and his alcoholism. He had begun drinking heavily in 1948 while still in graduate school, and by 1950 he was drinking in the morning. In 1951 O'Neill's wife's uncle, an early member of AA, had suggested the possibility of alcoholism. But his wife insisted to the Grant Study, with whom she had maintained connection even while O'Neill did not, that her husband was not abusing alcohol. Furthermore, in 1952, at his first admission, the health services at his university whitewashed his drinking as due to "combat fatigue." His prospective 1946 military record revealed, however, that O'Neill had experienced no combat in World War II.

In 1972 I interviewed O'Neill, and he filled in some long-standing gaps. We met in his apartment. He was balding and sported a distinguished mustache; his clothes were worn but elegant. He came across as an energetic man who kept a tight rein on his feelings. At first during the interview he had a lot of trouble looking at me and seemed very restless. He chain-smoked, walked back and forth, lay down first on one bed and then on the other. Although he avoided eye contact, there was a serious awareness of me as a person, and I always felt he was talking to me. He behaved like a cross between a diffident professor and a newly released prisoner of war. As he put it to me, "I'm hyper-emotional; I'm a very oversexed guy. The feelings are there, but it's getting them out that's hard. The cauldron is always bubbling. In Alcoholics Anonymous, I'm known as Dr. Anti-Serenity."

He admitted that he had been chronically intoxicated between 1952 and 1955 while writing his Ph.D. thesis, and that he had regularly sold books from the university library to buy alcohol. By 1954 his wife had begun to complain about his drinking; by 1955 it was campus gossip. But no diagnosis of alcoholism was made during his 1957 VA hospital admission or the subsequent one in 1962. In our 1972 interview, I felt that O'Neill himself still did not understand the cause-and-effect relationship between his drinking and his misery.

In 1970, O'Neill became sober in Alcoholics Anonymous. By our 1972 interview, AA was clearly the most important force in his life, besides his wife. He made frequent reference to it; when I asked him what his dominant mood was, he replied, "Incredulity. . . . I consider myself lucky. Most people in Alcoholics Anonymous do."

Even after two years of complete sobriety, O'Neill described himself to me as "a classical psychopath, totally incapable of commitment to any man alive." To me, though, he felt like a lonely but kindly man. I never had the feeling that he was cold or self-absorbed. If anything, he suffered from a hypertrophy of conscience, not a lack thereof. Remember that, although alcohol does not help insomnia, chronic anxiety, or depression, it is the best antidote for guilt that we have.

As I was leaving, I noticed several books related to gambling on the bookshelves. Aha, I thought. Were these the lingering remnants of the sociopathy he talked about? No, as it turned out. Once sober, he had sublimated his interest in gambling. He had consulted to the governor of Louisiana while the state lottery there was being set up—a considerably more profitable occupation for an economist than frequenting racetracks. In other words, with the remission of his alcoholism, O'Neill's ego functioning had matured; instead of compulsively acting out his interest in gambling, he had yoked his passion to his Ph.D. in economics, and harnessed them both in a socially and personally constructive way.

In closing, O'Neill told me that he could not agree with AA in calling alcoholism a disease. "I think that I will the taking up of a drink," he said. "I have a great deal of shame and guilt and remorse and think that's healthy." I heartily disagreed; I suspect that his shame had facilitated his denial of his alcoholism for twenty years, and that by reframing it as a disease, AA had rescued him. Sadly, O'Neill died two years after our interview from coronary heart disease, a fate undoubtedly hastened by twenty-five years of chain smoking.

THE STORY OF FRANCIS LOWELL

The history of Francis Lowell, aka Bill Loman, illustrates how different alcoholism looks to sociologists and to physicians. In this case, the two viewers were myself! Without realizing it, I narrated one man's story twice, never recognizing him the second time as a person I had studied fifteen years before.

Francis Lowell was an effective and well-paid upper-class New York lawyer. In 1995 I used his life as evidence that the misuse of alcohol, like heavy smoking, is not a disease, but a lifestyle choice. ³² Given enough education, willpower, social support, and a forgiving occupation, a fortunate drinker could drink as long and as much as he wanted. In college Lowell had been a heavy social user of alcohol, and very guarded about answering Grant Study questions related to his alcohol use. By age twenty-five, this gregarious man had established a pattern of heavy drinking Friday through Sunday, and none during the rest of the week. He continued this pattern over the next forty years. His heavy weekend drinking sometimes expanded into five-day binges, with a loss of one or two days of work, but Lowell abused alcohol from age thirty to age seventy without any noticeable decline in his physical health or serious damage to his legal career (most of his clients were rich family members).

Lowell was aware that he had a problem with his drinking by the time he was thirty. He felt guilty about how much he drank; his friends criticized his drinking; he failed to keep his promises to cut down; and when he was drinking he avoided his relatives. At age thirty-nine he had his first drunk-driving arrest; there was a second one at age forty-seven. He had his only detoxification at age fifty-two, but his physical exam and liver chemistries were normal. At age fifty-six, Francis Lowell said of himself, "No doubt about it, I do drink heavily at times," but he never stopped drinking, except for giving it up for Lent. Many weeks he drank within social limits, and usually he did not drink during the week. He attributed his successful steady pattern of alcohol abuse to the fact that his stomach would not tolerate more than five days of drinking. And, he added, "I don't want to sound pompous, but a sense of duty drilled into me from family and from St. Paul's School contributes to my control. . . . You just can't let everything go."

By fifty-nine, Francis Lowell was making \$200,000. His heavy alcohol intake did not interfere with his work, although his career did not advance after sixty. And it did not (much) interfere with his relationships; it had contributed to his loss of the woman who had most touched his heart, but by remaining single he limited further damage. After he turned sixty-two, his doctor began encouraging him to cut down on his drinking, and at age sixty-six he had a seizure "possibly related to alcohol." Nevertheless, at seventy Francis Lowell was still working forty hours a week and earning his handsome salary. Compared to his college classmates he was still very active physically, and his liver chemistries were still normal. I wrote in my first biography of him, "At no time in his life has he described a wish to become abstinent and he continued to drink ten drinks a day on the weekend." In short, I believed that Francis Lowell had a lifelong problem with alcohol, but not a "progressive disease."

THE STORY OF BILL LOMAN

But alcoholism has an unstable, chameleon-like quality. After I had forgotten my original description of Lowell, I wrote the life history of a chronic alcoholic whom I called Bill Loman. It wasn't until after the fact that I discovered that I had written about the same man in 1983, seeing his life from a very different point of view, and making a point very different from the one I was aiming at the second time. Although there had been only four more years of follow-up and a

little bit more data, my view of Lowell/Loman had shifted from the sociological to the medical one. Light can act as both wave and particle, and alcoholism can present as both habit and disease. Only years of observation allow us to identify both of these patterns in the same person. Bill Loman was one of the Study's great illustrations that the genes for alcoholism can derail anyone, no matter how promising his beginnings.

Bill Loman was a man destined for great things; he became a tragic figure not because he deserves our scorn, but because he had the disease of alcoholism as his implacable enemy. At St. Paul's School, Loman had been a senior prefect and captain of the football team. He was elected to the most prestigious club at Harvard, and he graduated magna cum laude. College descriptions of Loman included "unspoiled by his wealth," "well-poised and very attractive," "rather mature."

His World War II record was exemplary too. He won three battle stars for active participation in the Battle of the Bulge and the crossing of the Roer and the Rhine Rivers. His commanding officer described him as "intensely loyal, collected and cool under most trying conditions. . . . Sense of humor never deserts him." He was promoted to first lieutenant and then to captain. The Study director, summing up the twenty-five-year-old Loman's military record, remarked, "This boy could go quite far." After the war, Loman went to Harvard Law School and finished in the top tenth of his class. He returned to New York to practice corporate law at a prestigious firm. He was elected to the very best clubs in the city, and spent his weekends playing golf and bridge with other members. At thirty, he was an upper-class football captain poised to be a superstar when he grew up.

But he didn't. There was another thread running through Bill Loman's life, and it could be seen even while he was still in college. He spent his weekends drinking. In college he experienced three- and four-day episodes of "depression"—probably alcohol-related—when he would see the world as a "very sorry place." The college psychiatrist saw the hard-drinking Loman as "undependable, careless, self-centered and evasive." In the military, too, Loman recalled, "I spent most of my spare time drinking and chasing women."

In law school, Loman dared to drink only on weekends—he already recognized that he could not have even one drink without going on a binge. By the age of thirty he had established a pattern of drinking heavily from lunch on Friday through Sunday evening. He was abstinent during the week, but he took frequent sick days, especially on Monday when he was recovering from weekend hangovers.

Intimacy, Career Consolidation, and Generativity were not in Bill Loman's future. He became emotionally involved with only one woman, in his twenties. At thirty he proposed to her, but she turned him down. Pressed, he admitted that her reluctance to marry him might have been due to his binge drinking. They remained closely involved for the next twenty-five years, both living with their mothers on the weekends. When he was fifty-three, her "dominating" mother died, and she married someone else. Loman continued to live with his mother until she died. He was discontented throughout his legal career, feeling undercommitted and undercompensated.

I interviewed Bill Loman when he was fifty-nine. He was insecure and unable to make eye contact. Instead of enjoying the interview as most Study members did, he acted like an unhappy adolescent being grilled. "I don't think I'd have joined the Study if I knew it was going to last so long," he grumbled.

Loman manifested a pervasive melancholy. When he was fifty, his brother, another Study man who did grow up to be a superstar, sadly revealed that Bill had stopped making new friends. Despite his high income, he took no vacations, involved himself in no civic activities, and had no exciting relationships with the opposite sex. There had never been anyone in whom he had confided. Asked who he turned to in unhappiness, Loman replied, "I turn to me."

At age sixty-five when asked, "What is your most satisfying activity?" Loman answered, "None." Unlike many unhappy Study men and women who could rely on strong religious affiliations even when socially isolated, Loman went to church just once a year—on Christmas, with his mother. He had been raised in a church school. But alcohol abuse interferes with spiritual solace as well as the more mundane kinds.

After our interview, Loman's alcoholism continued to progress. By sixty-five, despite reasonable health, the once-sociable Bill no longer attended any of his clubs. He could not learn new things. Unlike most of his Harvard cohort, he never even began to master the computer. The personal losses that accrue in life had never been replaced; among his friends and relatives there was no longer a single person with whom he could say he had an intimate relationship. Not

surprisingly, he saw the present as the unhappiest period of his life. The happiest time, he said, had been the war years. It was hard not to be disconcerted by that. The Battle of the Bulge is not everyone's idea of a day at the beach.

Loman's alcoholism never affected his liver, but it destroyed his life. At thirty, both he and the girl he loved were already worrying about his drinking. At age forty, after three DWI's with damage to others, his mother, his brother, and the police were worrying too. At age fifty-three he sought his first detoxification. But at sixty he was still going on binges where he'd drink a quart of whiskey a day for five days. The only thing that stopped him was that he always eventually became too ill to drink anymore. He had his first seizure due to alcohol withdrawal at sixty-five.

After that Loman made repeated unsuccessful efforts to go on the wagon until he died from an alcohol-related fall at seventy-four. Seventy percent of his Study mates were still alive. He did not begin his life among the Loveless, but he ended it that way.

My readers will have noted, I am certain, the—unconscious—shift in focus, and even in detail, between these two versions of Lowell/Lo-man's life. As I've said before, biography is more vivid than statistics, but it is much more vulnerable to vicissitudes in the writer's intent than numbers are. The fact is that both Lowell and Loman had a problem drinking score (PDS) of 11—in the top 10 percent of both the College and Inner City samples. For many years I struggled to make sense out of conflicting views of alcoholism, both of which had fierce partisans: was it a disease or a career path? For most of his life, Lowell/Loman's lab tests and physical exams were clean. For most of his life his mates drank as much as he did. His career, in its own way, prospered. I can see now that he was the perfect example of why it's so hard to say what alcoholism "really is." It wasn't until his last years that the evidence of what alcohol had done to him physically and psychologically really began to show. Another reason for lifetime studies. Alcoholism is a crafty foe, and even under the intense scrutiny of the Grant Study, in the case of Lowell/Loman it managed to keep itself hidden for a very long time.

We can all consider for ourselves whether in my first effort to tell this story I was in my own kind of denial, missing the disease that is alcoholism, or whether in my second I was bending the facts, forcing a heavy habitual drinker into the mold of an alcoholic. But however we explain it, both Lowell and Loman suffered from something that was indeed cunning, baffling, and powerful. It was also ultimately fatal.

CONCLUSION

Prospective study has consistently shown alcoholism to be the cause, not the result, of dependent, sociopathic, neurotic, or aggressive personality disorders. Alcoholism is the cause, not the result, of unhappy marriages. Alcoholism is the cause of many deaths, too, and not only through liver cirrhosis and motor vehicle accidents—suicides, homicides, cancers, heart disease, and depressed immune systems can all be chalked up to this serial killer. The critical factors predicting recovery from alcohol dependence, besides its severity, appear to be finding a (preferably) nonpharmacological substitute for alcohol, compulsory supervision (with immediate negative consequences for relapse), new loving relationships, and involvement in inspirational programs.

Prolonged follow-up reveals two fundamental paradoxes in predicting the life course of alcoholism. Socially disadvantaged men, men with strong family histories of alcoholism, and men with early onset of severe alcohol dependence were more likely than other men to become stably abstinent. In contrast, alcohol abusers with excellent social supports, high education, good health habits, and late onset of minimal alcohol abuse—epitomized by the College sample—were more likely to remain chronic alcohol abusers. If their alcohol-related problems were truly minimal, they also had an excellent chance of returning to lifelong social (controlled) drinking. In short, it appears to be the most and the least severe alcoholics who enjoy the best chance of long-term remission.